# The introduction film

MedLYFE. First shown at AGELESS

| | |
|---|---|
| FOR | The production company |
| FROM | MedLYFE Wellness and Longevity Centre, with Consult for Africa |
| DATE | Friday 9 October 2026 |

## In short

- One film, 75 to 90 seconds. It introduces MedLYFE, and it has to keep doing that for months.
- First outing is 6:45pm on Wednesday 21 October at Capital Club, Lagos, as the evening opens.
- After that it is the standing introduction: the website hero, the first thing on every discovery call, reception, the sales deck, the WhatsApp forward.
- So nothing in it is about the event. No date, no venue, no guests, no AGELESS. It must not expire on the 22nd.
- It carries the discovery deck's spine, not a new story. Same argument, same words, so the film and the call agree.
- Half a day at the centre. One location, three setups, one person to camera.
- Shoot Tuesday 13 or Wednesday 14 October. Final file by Monday 19 October.
- There is a second block that can ride the same production. See below.
- No patients. No before and after. No drug brand names. The section on what must not be in it is not negotiable.

## Two blocks, one production

The shoot briefed for Thursday 8 October did not go ahead, so the library it was to produce does not exist and the crew is not committed. Dr Chinwe Kpaduwa is in Lagos for the evening on the 21st, which gives a window that did not exist a week ago.

| | What | When |
|---|---|---|
| **A** | The opening film. This document | Fixed. Delivered Monday 19 October |
| **B** | Dr Kpaduwa's educational library, as briefed on 6 October | Movable. Around her availability, before or after the 21st |

**Block A is the one with a deadline, and it is the one to quote first. If taking both at once costs you the 19 October delivery, do not take both. The film is cued in a room on a fixed date and the library is not.**

If you can take both, say what the combined day or days look like and what it saves against quoting them apart. One crew call, one kit hire and one grade across two blocks should be worth something to us both, and we would rather you told us the number than discovered it later.

Block B's deliverables, locations and conditions are unchanged from the brief of 6 October. Nothing in this document replaces it.

## The deadline governs everything

The film is cued in a room on a fixed date. There is no version of this where it slips.

| | |
|---|---|
| **Brief issued** | Friday 9 October |
| **Confirm you can crew it** | Saturday 10 October |
| **Recce, in person or by video** | Monday 12 October |
| **Shoot** | Tuesday 13 or Wednesday 14 October |
| **First cut** | Friday 16 October |
| **Notes back to you** | Saturday 17 October |
| **Final file delivered** | Monday 19 October |
| **Technical check at Capital Club** | Tuesday 20 October |
| **Event** | Wednesday 21 October |

If you cannot hold Monday 19 October, say so on Saturday. That is as useful to us as a yes.

## What the film has to do

Somebody meets MedLYFE for the first time. Ninety seconds later they should hold one idea: there is a gap between how long you live and how well you live, and this is a medical centre that measures you and closes it.

That is the discovery deck's own argument, and the film does not invent a second one. A prospect who watches the film and then sits through the call should hear the same thing twice, in the same words. Two different stories is how a centre sounds improvised.

- It is not a clinic tour. Rooms are the setting, not the subject.
- It is not a list of services. The pyramid has six layers and naming them all in ninety seconds reads as a menu.
- It is not aspirational stock. No sunrises, no yoga on a beach, no stethoscope on a desk.
- It is not about the evening. It opens AGELESS and hands to Dr Ajelabi, but nothing in the picture, the words or the end card refers to the event. It has to be as right in March as it is on the 21st.

## The film

Ninety seconds, give or take. Picture and sound against the clock.

The lifespan and healthspan bars on slide two of the discovery deck are the one graphic in the film. Animate them: two lines from birth, one running to the end, one stopping short, and the gap between them opening up. That gap is the whole proposition and it is worth eight seconds.

| Time | Picture | Sound |
|---|---|---|
| **0:00** | Black. The MedLYFE mark resolves, small, centred | Room tone, then a low sustained note |
| **0:04** | Dr Ajelabi, seated, to camera, daylight | Line one |
| **0:14** | The two bars animate. Lifespan runs on. Healthspan stops short. The gap opens | Music enters |
| **0:22** | Dr Ajelabi, to camera | Line two |
| **0:30** | Close: a blood sample into an analyser. Hands, gloves, no faces | Music |
| **0:36** | Close: real numbers appearing on a screen. Readouts, not motion graphics | |
| **0:42** | Dr Ajelabi, to camera | Line three |
| **0:54** | The infusion room, empty, in daylight. Slow push | Music lifts |
| **1:00** | Close: a measurement being taken on a colleague, not a patient | |
| **1:06** | Dr Ajelabi, to camera | Line four |
| **1:16** | Hold on her. She does not smile on cue | Music resolves |
| **1:20** | Card: MedLYFE Wellness and Longevity Centre. Live Better for Longer | |
| **1:26** | Black | Out |

### What she says

Four lines, learned rather than read. No autocue if it can be avoided; an earpiece is better than a wandering eye. Three of the four are lifted from the discovery deck on purpose.

> One. Lifespan is how long you live. Healthspan is how well you live.

> Two. For most people there is a gap at the end. Years spent in decline. Closing that gap is the whole of our work.

> Three. We don't guess. We test, then we act.

> Four. We do not prescribe transformation before we understand the person.

If she would rather say it in her own words, that is better than these words, with one exception: lines three and four are MedLYFE's own and should survive intact. They are the two sentences in the deck that sound like a position rather than a brochure.

## On camera

| Who | What |
|---|---|
| **Dr Adedotun Ajelabi** | Head of Medicals. The only person who speaks |
| **Two colleagues** | Hands and backs of heads only, for the clinical inserts. Staff, not actors, and not patients |

Nobody else is on camera. A second clinician on screen for four seconds costs an hour of the day.

## Location and setups

One location, the MedLYFE centre. Three setups, which is what keeps this to half a day.

| Setup | Where | For |
|---|---|---|
| **A** | A room with daylight, seated | Dr Ajelabi's three lines. The only sync sound in the film |
| **B** | The laboratory or diagnostics bench | The analyser, the sample, the screen, the measurement |
| **C** | The infusion room, and one wide architectural frame | The empty room and the exterior or atrium |

If the building will not give you a wide architectural frame that flatters it, tell us at the recce and we will cut that shot rather than shoot a bad one.

## Where it lives after the 21st

The event is the first outing, not the job. This is what the film is for, and it is why the versions below exist.

| Where | Which version |
|---|---|
| **Capital Club, 21 October** | The master, projected |
| **medlyfewellness.com, above the fold** | The muted hero loop. There is no video on the site today |
| **The first ninety seconds of every discovery call** | The master, with sound |
| **Reception, on a loop** | The muted hero |
| **Inside the sales deck** | The master, embedded |
| **Instagram, TikTok, WhatsApp** | Vertical and square, captioned |

A film that does six jobs is worth six times a film that does one, which is the argument for shooting it properly rather than cutting something from stills.

## Deliverables

| | Specification |
|---|---|
| **The master** | 1920 x 1080, H.264, 25fps, stereo, loudness normalised. Delivered as a single named file. This is the one that gets projected on the 21st |
| **Website hero, muted** | 1920 x 1080, no audio track, graded to sit behind text. Under 4MB, so it loads on Lagos 4G |
| **Vertical** | 1080 x 1920, burned-in captions |
| **Square** | 1080 x 1080, burned-in captions |
| **A 30 second cut** | From the same material, 16:9 and 9:16, for paid social and for a cold audience who will not give you ninety seconds |
| **Stills** | 15 or more frames, retouched, full and web resolution |
| **Project files and masters** | Every rush, plus the project, on physical media. We will want to re-cut this in six months without re-shooting |

Captions are burned in, not supplied as a sidecar. Most of this is watched with the sound off and forwarded on WhatsApp.

## What must not be in it

This is the part that carries a fine, so it is written out rather than assumed.

- No patients. Nobody who could be read as a patient appears in any frame. Staff only, and only as hands and backs of heads.
- No before and after. None filmed, none supplied, none used.
- No drug or product brand names. NAFDAC does not permit prescription medicines to be advertised to the public by name. Say what something does, never what it is called.
- No claim of a result. Nothing that promises an outcome, a timescale or a cure.
- No comparison to another clinic, named or implied.
- Dr Ajelabi speaks for the centre, not for herself. The MDCN treats a practitioner advertising themselves as a disciplinary matter. She is introduced by role, and the centre is the advertiser.

One further point, for the website version only. A film shown to an invited room on one evening is not the same thing as a film running on a public website. The website version is an advertisement and sits inside ARCON's remit, which means pre-exposure approval and a licensed agency to file it. Liability is on the advertiser, not on you. Flag it to us before the hero version goes live rather than after.

## What we provide

- Access to the centre for the shoot day, and for the recce on Monday 12 October.
- Dr Ajelabi, for a maximum of two hours inside the shoot day.
- Two staff for the clinical inserts.
- The MedLYFE mark, brand colours and the end card specification.
- The discovery call deck, which is where the argument and two of the four lines come from. Read it before the recce.
- A named person on set who can approve a shot on the spot, so the day does not stop for a telephone call.

## Please quote against these lines

Itemised, so pre-production, the day and post can be weighed separately.

| | |
|---|---|
| **1** | Pre-production, recce and treatment |
| **2** | Director and crew, half day |
| **3** | Camera, lenses and support |
| **4** | Lighting and grip |
| **5** | Sound |
| **6** | Edit, to two rounds of notes |
| **7** | Grade and online |
| **8** | Sound mix and music licence |
| **9** | Motion graphics: mark resolve, the lifespan and healthspan animation, lower third, end card |
| **10** | Versioning: the master, the muted hero, vertical, square and the 30 second cut |
| **11** | Media, delivery and archive |
| **12** | Anything you believe is missing from this list |

Quote the half day as a half day. If you can only crew a full day, quote that as well and we will decide.

Then, separately and underneath, quote Block B against the lines in the brief of 6 October, and give us the saving if both are taken as one production.

## Rights

Full, perpetual, worldwide, across every medium, to MedLYFE Wellness and Longevity Centre. Music cleared on the same basis, with the licence supplied.

You are welcome to use the finished film in your own portfolio and showreel, and we will credit you where there is a credit to give.

# What we would like back on Saturday

1. Whether you can crew Tuesday 13 or Wednesday 14 October.
2. Whether you can deliver the event master by Monday 19 October.
3. The quotation for Block A, and separately for Block B.
4. Whether taking both as one production changes either the price or the 19 October date.
5. Anything in this brief you would do differently. We would rather hear it now than see it in the cut.